Must Love Scrubs — The NGN Workbook

50 Original Next Gen NCLEX-Style Questions

With a rationale for every option, an NCLEX tip, and a memory trick.

M❤️‍🩹S · Must Love Scrubs
Education with a pulse.

How to use this workbook

This is not a bank of memorized answers — it's a thinking workbook. For each question:

1. **Read the scenario and answer before you look down.** Commit to a choice.
2. **Check the illustration note.** In the app, every question ships with the art described in the 🎨 line — use it to picture the scene.
3. **Read the rationale for *every* option, not just the right one.** The wrong answers are where the NCLEX hides its lessons. Knowing *why* a good-looking answer is wrong is what separates passing from guessing.
4. **Steal the tip and the trick.** The 📝 NCLEX Tip tells you what the test is really asking. The 💡 Pearl gives you a memory hook for test day.

Sections

Section 1 · Pharmacology & Physiological Adaptation

Question 1 — Anticoagulation teaching

Category: Pharmacology · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A 72-year-old client with chronic atrial fibrillation is being discharged home on warfarin. The nurse is reinforcing discharge teaching.

Which client statement indicates that teaching has been effective?

- **A.** "I'll double my dose if I ever miss a day."
- **B.** "I'll keep the amount of leafy green vegetables I eat about the same each week."
- **C.** "I can take ibuprofen for my arthritis whenever I need it."
- **D.** "I'll stop the medication as soon as my heartbeat feels regular."

🎨 *Suggested illustration:* a plate split into consistent weekly portions of spinach and kale beside a warfarin bottle, with a small INR lab slip in the corner.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Doubling a dose to "catch up" can push the INR into a dangerous range and cause bleeding. A missed dose is reported to the provider, not doubled.
- **B — Correct.** Warfarin works by antagonizing vitamin K. Green leafy vegetables are high in vitamin K, so the goal is **consistency**, not avoidance — a steady weekly intake keeps the INR stable.
- **C — Incorrect.** NSAIDs like ibuprofen increase bleeding risk (antiplatelet effect plus GI irritation) and should not be taken freely with warfarin.
- **D — Incorrect.** Warfarin is prescribed to prevent clot formation in atrial fibrillation regardless of how the rhythm *feels*; stopping it abruptly raises stroke risk. Clients do not self-discontinue.

📝 **NCLEX Tip.** When a question offers "avoid all vitamin K foods," it's a trap. The correct anticoagulation teaching is almost always **consistency**, not elimination.

💡 **Clinical Pearl / Memory Trick. W**arfarin → **W**atch the **INR**; antidote is **vitamin K**. Heparin → watch the **aPTT**; antidote is **protamine**. "K for Koumadin, P for heParin."

Question 2 — Digoxin toxicity

Category: Pharmacology · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A nurse is caring for an older adult who takes digoxin and furosemide for heart failure. The client reports nausea, "yellow-green halos" around lights, and no appetite.

What is the nurse's priority action?

- **A.** Administer the next scheduled dose of digoxin as ordered.
- **B.** Hold the digoxin and obtain a serum digoxin and potassium level.
- **C.** Encourage a high-potassium breakfast and reassess in an hour.
- **D.** Document the findings as expected side effects of heart failure.

🎨 *Suggested illustration:* a client squinting at a lamp ringed with a yellow-green halo, a med cup being set aside, and a lab tube labeled "dig level."

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Giving more digoxin to a client showing classic toxicity signs could be fatal. Never administer the next dose until toxicity is ruled out.
- **B — Correct.** Nausea, anorexia, and **visual disturbances (yellow-green halos)** are hallmark signs of digoxin toxicity. The nurse holds the drug and checks the digoxin level *and* potassium, because **hypokalemia** (which furosemide can cause) potentiates digoxin toxicity.
- **C — Incorrect.** Diet alone won't correct active toxicity, and delaying assessment risks a lethal arrhythmia.
- **D — Incorrect.** These are not expected heart-failure findings — they are a medication emergency that must be acted on.

📝 **NCLEX Tip.** If a client on digoxin *also* takes a loop or thiazide diuretic, the test wants you to connect **low potassium → higher toxicity risk.**

💡 **Clinical Pearl / Memory Trick.** "**Dig** makes you **sick, slow, and see yellow**": GI upset, **brady**cardia, and yellow/green vision. Hold for an apical pulse **< 60**.

Question 3 — Loop diuretic monitoring

Category: Pharmacology · **Client Need:** Physiological Adaptation · **Difficulty:** Easy

Scenario. A client is started on IV furosemide for pulmonary edema.

Which laboratory value is most important for the nurse to monitor?

- **A.** Serum sodium
- **B.** Serum potassium
- **C.** Serum calcium
- **D.** Serum magnesium

🎨 *Suggested illustration:* a labeled electrolyte panel with the potassium (K^+) row circled in coral, and a downward arrow beside it.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Furosemide can lower sodium, but hyponatremia is generally less immediately dangerous than the potassium loss it causes.
- **B — Correct.** Furosemide is a **potassium-wasting** loop diuretic. Hypokalemia is the priority concern because it can trigger life-threatening cardiac dysrhythmias — especially if the client also takes digoxin.
- **C — Incorrect.** Loop diuretics can slightly lower calcium, but this is not the priority monitoring parameter.
- **D — Incorrect.** Magnesium can drop with loop diuretics and matters, but **potassium** is the classic, highest-priority answer the test is looking for.

📝 **NCLEX Tip.** Match the diuretic to the electrolyte: **loop and thiazide → lose potassium; spironolactone → keep (raise) potassium.**

💡 **Clinical Pearl / Memory Trick.** "**F**urosemide → **F**alling K." Normal potassium is **3.5–5.0 mEq/L** — memorize this range cold; it shows up everywhere.

Question 4 — Insulin onset and peak

Category: Pharmacology · **Client Need:** Pharmacological Therapies · **Difficulty:** Moderate

Scenario. A client receives regular (short-acting) insulin at 0730 before breakfast.

At which time should the nurse be most alert for signs of hypoglycemia?

- **A.** 0745
- **B.** 1000
- **C.** 1400
- **D.** 1900

🎨 *Suggested illustration:* a simple time-vs-glucose curve for regular insulin peaking around 2–3 hours, with a red "watch zone" band over the peak.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Regular insulin's onset is about 30 minutes; at 15 minutes it has barely begun working.
- **B — Correct.** Regular insulin **peaks about 2–3 hours** after administration. Given at 0730, its peak — and the highest hypoglycemia risk — falls around **1000**, which is why a mid-morning snack is often timed here.
- **C — Incorrect.** By 1400 (6+ hours) regular insulin's effect is waning.
- **D — Incorrect.** 1900 is far outside regular insulin's duration (about 5–8 hours).

📝 **NCLEX Tip.** For any insulin question, ask: *when does it peak?* — that's when the client is most likely to go low. Peak = highest risk.

💡 **Clinical Pearl / Memory Trick.** "**R**egular is **R**eady in a half hour, **R**oars at ~2–3." Rapid (lispro/aspart) peaks in ~1 hour — give it **with food in front of the client.**

Question 5 — Acute transfusion reaction

Category: Pharmacology / Safety · **Client Need:** Pharmacological Therapies · **Difficulty:** Moderate

Scenario. Fifteen minutes into a unit of packed red blood cells, a client develops chills, flank pain, and a temperature rise from 37.0°C to 38.6°C.

What is the nurse's first action?

- **A.** Slow the transfusion and continue to monitor.
- **B.** Stop the transfusion and keep the vein open with normal saline.
- **C.** Administer acetaminophen and reassess in 30 minutes.
- **D.** Notify the blood bank to prepare the next unit.

🎨 *Suggested illustration:* a hand clamping the blood tubing while a second line of normal saline runs into the same catheter; a thermometer reading 38.6°C.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Slowing the transfusion still infuses incompatible blood. Any acute reaction means the blood must **stop**, not just slow.
- **B — Correct.** The priority in a suspected acute hemolytic reaction is to **stop the blood immediately** and **maintain IV access with normal saline through fresh tubing** to preserve the line and support the kidneys. Then notify the provider and blood bank.
- **C — Incorrect.** Medicating and waiting delays life-saving action during a possible hemolytic reaction.
- **D — Incorrect.** Preparing another unit is premature and unsafe until the reaction is investigated.

📝 **NCLEX Tip.** For *any* transfusion reaction, the first step is always the same: **STOP the blood.** Then saline, then assess, then notify. Order matters.

💡 **Clinical Pearl / Memory Trick.** "**Stop – Saline – Stay – Send**": Stop the unit, hang **Saline**, **Stay** with the client and assess, **Send** the blood and tubing back to the blood bank.

Question 6 — Lithium therapy

Category: Pharmacology / Mental Health · **Client Need:** Pharmacological Therapies · **Difficulty:** Moderate

Scenario. A client with bipolar disorder takes lithium. During a summer heat wave, the client reports vomiting, coarse hand tremors, and slurred speech.

Which action should the nurse take first?

- **A.** Reassure the client that these are normal, expected effects of lithium.
- **B.** Hold the next dose and obtain a serum lithium level.
- **C.** Encourage the client to restrict all fluids and salt.
- **D.** Increase the lithium dose to control the tremor.

🎨 *Suggested illustration:* a sweating client under a bright sun holding a trembling hand out, with a lab slip reading "lithium 1.8 mEq/L" flagged in red.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Coarse tremors, vomiting, and slurred speech are signs of **lithium toxicity**, not benign side effects (a fine hand tremor can be expected; a *coarse* tremor is not).
- **B — Correct.** These findings suggest toxicity, which is more likely when dehydration or sodium loss concentrates the drug (as in a heat wave). The nurse **holds the dose and checks the level.** Therapeutic range is narrow: **0.6–1.2 mEq/L.**
- **C — Incorrect.** Restricting fluids and salt *worsens* lithium toxicity. Adequate, consistent sodium and fluid intake keep the level stable.
- **D — Incorrect.** Adding drug to a toxic client is dangerous.

📝 **NCLEX Tip.** Lithium and sodium travel together in the body. **Low sodium or low fluid = higher lithium = toxicity.** Watch for sweating, vomiting, and diuretics.

💡 **Clinical Pearl / Memory Trick.** Lithium range **"0.6 to 1.2, keep it true."** Early toxicity: **GI upset + fine tremor**. Worsening toxicity: **coarse tremor, slurred speech, confusion.**

Question 7 — Magnesium sulfate in preeclampsia

Category: Pharmacology / Maternity · **Client Need:** Pharmacological Therapies · **Difficulty:** Hard

Scenario. A client at 34 weeks' gestation with severe preeclampsia is receiving IV magnesium sulfate. The nurse notes a respiratory rate of 10/min, absent deep tendon reflexes, and urine output of 20 mL over the last hour.

What is the nurse's priority action?

- **A.** Increase the magnesium sulfate infusion rate.
- **B.** Stop the infusion and prepare to administer calcium gluconate.
- **C.** Place the client in a supine position and continue the infusion.
- **D.** Document the findings and reassess in one hour.

🎨 *Suggested illustration:* an IV pump paused, a reflex hammer with a "no response" symbol at the knee, and a vial of calcium gluconate ready at the bedside.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Increasing the rate would deepen magnesium toxicity and could stop respirations.
- **B — Correct.** Respiratory depression, **absent deep tendon reflexes**, and **oliguria** are signs of magnesium toxicity. The nurse **stops the infusion** and gives the antidote, **calcium gluconate.**
- **C — Incorrect.** Supine positioning risks aortocaval compression and does nothing about the toxicity; the infusion must stop.
- **D — Incorrect.** Waiting an hour during respiratory depression is unsafe.

📝 **NCLEX Tip.** Before and during a magnesium infusion, the test wants you checking three things: **respirations (≥12), deep tendon reflexes (present), and urine output (≥30 mL/hr).** Lose any one → suspect toxicity.

💡 **Clinical Pearl / Memory Trick.** "**Mag** makes it all **sag**" — reflexes, respirations, and BP drop. Antidote rhymes: **"Mag toxicity? Give calcium gluconate quickly."**

Question 8 — Oxygen therapy in COPD

Category: Pharmacology / Respiratory · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A client with chronic COPD is admitted with shortness of breath. The provider orders oxygen. The client's baseline SpO_2 runs 88–92%.

Which oxygen delivery is most appropriate for the nurse to initiate?

- **A.** Non-rebreather mask at 15 L/min
- **B.** Nasal cannula at 2 L/min, titrated to keep SpO_2 88–92%
- **C.** Room air only, because oxygen is unsafe in COPD
- **D.** Simple face mask at 10 L/min

🎨 *Suggested illustration:* a nasal cannula on a calm client with a pulse-ox reading 90%, next to a small dial showing "2 L/min."

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** High-flow oxygen can be appropriate in an emergency, but as a routine start in stable COPD it may blunt the hypoxic drive and is more than needed to reach the target.
- **B — Correct.** Clients with chronic COPD are treated to a **target SpO_2 of about 88–92%**, not 100%. Low-flow oxygen titrated to that range relieves hypoxia without over-oxygenating.
- **C — Incorrect.** Withholding oxygen from a hypoxic client is dangerous — hypoxia will harm them long before the theoretical drive concern does.
- **D — Incorrect.** A simple mask at 10 L/min delivers far more oxygen than the target requires.

📝 **NCLEX Tip.** For chronic COPD the safe goal is a **lower** SpO_2 target (88–92%). Beware answers that push COPD clients to 100% — over-oxygenation is the trap.

💡 **Clinical Pearl / Memory Trick.** "In COPD, **low and slow**": low-flow O_2, target in the **high 80s to low 90s.** Never *withhold* oxygen from a hypoxic person — treat the hypoxia, just don't overshoot.

Question 9 — Beta-blocker teaching

Category: Pharmacology / Cardiac · **Client Need:** Pharmacological Therapies · **Difficulty:** Easy

Scenario. A client newly prescribed metoprolol asks the nurse how to take it safely at home.

Which instruction should the nurse include?

- **A.** "Stop the medication right away if your heart rate feels slow."
- **B.** "Check your pulse before each dose and hold it if it's below 60."
- **C.** "Rise quickly from sitting to avoid getting dizzy."
- **D.** "You can stop taking it once your blood pressure is normal."

🎨 *Suggested illustration:* a client taking a radial pulse with two fingers while looking at a watch, a pill bottle labeled "metoprolol" nearby.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Beta-blockers should **never be stopped abruptly** — rebound tachycardia, hypertension, and even ischemia can result. They are tapered.
- **B — Correct.** Beta-blockers slow heart rate, so clients are taught to **check the pulse and hold the dose for a rate below 60** (per parameters) and notify the provider.
- **C — Incorrect.** Beta-blockers can cause orthostatic hypotension; clients should rise **slowly**, not quickly.
- **D — Incorrect.** The medication controls blood pressure only while taken; stopping it lets pressure climb back up.

📝 **NCLEX Tip.** Any drug ending in **"-olol"** is a beta-blocker: expect teaching about **checking the pulse, rising slowly, and never stopping abruptly.**

💡 **Clinical Pearl / Memory Trick.** "**Beta blocks the beats**" — it lowers **heart rate and BP.** Hold for a pulse **< 60.** Also masks the shakiness/ tachycardia of **hypoglycemia**, so diabetics need extra caution.

Question 10 — Opioid administration safety

Category: Pharmacology / Safety · **Client Need:** Pharmacological Therapies · **Difficulty:** Moderate

Scenario. A client recovering from surgery received IV morphine 20 minutes ago. The nurse finds the client difficult to arouse with a respiratory rate of 7/min.

Which action should the nurse take first?

- **A.** Administer the next scheduled dose of morphine.
- **B.** Stimulate the client, support breathing, and prepare to give naloxone.
- **C.** Place the client flat and darken the room to promote rest.
- **D.** Wait 15 minutes and reassess the respiratory rate.

🎨 *Suggested illustration:* a bedside scene with a nurse's hand on the client's shoulder, an oxygen bag-valve mask within reach, and a naloxone vial highlighted.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** More opioid to a client in respiratory depression could stop breathing entirely.
- **B — Correct.** A respiratory rate of 7/min with sedation signals **opioid-induced respiratory depression.** The nurse stimulates the client, supports ventilation/oxygenation, and prepares the antidote **naloxone (Narcan).**
- **C — Incorrect.** Lying flat and reducing stimulation would worsen sedation and hypoventilation.
- **D — Incorrect.** Waiting during respiratory depression risks respiratory arrest.

📝 **NCLEX Tip.** A respiratory rate **below 12/min** after an opioid is the red flag. The antidote **naloxone** is the answer whenever opioid over-sedation is described.

💡 **Clinical Pearl / Memory Trick.** "**N**aloxone **N**egates **N**arcotics." Watch the number: **RR < 12 = hold and reassess; RR ≤ 8 with sedation = act now.**

Section 2 · Adult Health / Med-Surg

Question 11 — Chest tube management

Category: Adult Health / Respiratory · **Client Need:** Reduction of Risk Potential · **Difficulty:** Moderate

Scenario. A client has a chest tube connected to a water-seal drainage system after a spontaneous pneumothorax. The nurse observes **continuous vigorous bubbling** in the water-seal chamber.

What does this finding most likely indicate?

- **A.** Normal, expected function of the drainage system
- **B.** An air leak in the system or tubing connections
- **C.** Complete lung re-expansion
- **D.** Obstruction of the chest tube

🎨 *Suggested illustration:* a three-chamber chest drainage unit with the water-seal chamber bubbling continuously, and a magnifying glass over a loose tubing connection.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** *Intermittent* bubbling in the water-seal chamber with respiration can be normal, but **continuous vigorous** bubbling is not.
- **B — Correct.** Continuous bubbling in the water-seal chamber signals an **air leak** — air is entering the system from a loose connection, a dislodged dressing, or the pleural space. The nurse checks all connections first.
- **C — Incorrect.** With full lung re-expansion, bubbling and tidaling **stop** — the opposite of this finding.
- **D — Incorrect.** An obstruction would reduce or stop drainage and tidaling, not cause continuous bubbling.

📝 **NCLEX Tip.** Learn the chambers: **water-seal = bubbling/tidaling info; suction-control = gentle continuous bubbling is normal there.** Continuous bubbling in the *water-seal* chamber = **air leak.**

💡 **Clinical Pearl / Memory Trick. "Tidaling is fine, continuous is a sign.**" If the tube is ever pulled out, cover with a sterile occlusive dressing taped on **three sides.**

Question 12 — Increased intracranial pressure

Category: Adult Health / Neuro · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client with a traumatic brain injury is being monitored for increased intracranial pressure (ICP).

Which set of findings should the nurse report immediately as an early sign of rising ICP?

- **A.** Restlessness and a decreasing level of consciousness
- **B.** Bradycardia, widening pulse pressure, and irregular respirations
- **C.** Hypotension and tachycardia
- **D.** Constricted, briskly reactive pupils

🎨 *Suggested illustration:* a brain in a fixed skull with pressure arrows pushing outward, and a small trend arrow showing LOC dropping first.

✅ **Correct answer: A**

Rationale
- **A — Correct.** The **earliest** and most sensitive sign of rising ICP is a **change in level of consciousness** — restlessness, confusion, or a subtle decline. Catching it early is what protects the brain.
- **B — Incorrect.** Bradycardia, widening pulse pressure, and irregular respirations (**Cushing's triad**) are **late, ominous** signs of severely elevated ICP — important, but not *early*.
- **C — Incorrect.** Hypotension and tachycardia point toward shock/hypovolemia, not classic ICP changes.
- **D — Incorrect.** Rising ICP tends to cause a **dilated, sluggish or fixed** pupil, not constricted and brisk.

📝 **NCLEX Tip.** Read carefully for **"early" vs "late."** Early ICP = **LOC change.** Late ICP = **Cushing's triad.** The same topic has two different "right" answers depending on that one word.

💡 **Clinical Pearl / Memory Trick. Cushing's triad = "high, low, slow, and odd"**: **high** systolic/**widening pulse pressure, low** heart rate (**slow**), and **odd/irregular** breathing. Keep the HOB at **30°** and the head midline to promote venous drainage.

Question 13 — Postoperative DVT prevention

Category: Adult Health / Perioperative · **Client Need:** Reduction of Risk Potential · **Difficulty:** Easy

Scenario. A client is on the first postoperative day after abdominal surgery and is reluctant to move.

Which nursing action best helps prevent deep vein thrombosis (DVT)?

- **A.** Massage the calves firmly twice per shift.
- **B.** Encourage early ambulation and leg exercises.
- **C.** Keep the knee gatch of the bed elevated continuously.
- **D.** Maintain strict bed rest until the client is pain-free.

🎨 *Suggested illustration:* a client taking first steps at the bedside with a nurse assisting, sequential compression sleeves visible on the calves.

✅ **Correct answer: B**

Rationale
- **A — Incorrect. Never massage** the legs of a client at risk for DVT — a clot could dislodge and become an embolus.
- **B — Correct. Early ambulation** plus ankle pumps and leg exercises promote venous return and are first-line DVT prevention, along with compression devices and prescribed anticoagulants.
- **C — Incorrect.** A continuously elevated knee gatch causes popliteal compression and venous pooling — it *increases* DVT risk.
- **D — Incorrect.** Prolonged immobility is a leading cause of DVT, not a prevention strategy.

📝 **NCLEX Tip.** When the answer choices include **"massage the calf,"** it's almost always the wrong (dangerous) option in an at-risk client.

💡 **Clinical Pearl / Memory Trick.** DVT prevention = **"Move, Squeeze, Thin"**: **Move** (ambulate/leg exercises), **Squeeze** (SCDs/compression), and **Thin** the blood (prophylactic anticoagulants). Don't rub.

Question 14 — Adrenal insufficiency (Addisonian crisis)

Category: Adult Health / Endocrine · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client with Addison's disease is admitted after several days of vomiting and diarrhea. The nurse notes blood pressure 82/50 mm Hg, weakness, and a serum sodium of 128 mEq/L.

Which finding requires the nurse's most immediate attention?

- **A.** Bronzed skin pigmentation
- **B.** Blood pressure of 82/50 mm Hg
- **C.** Craving for salty foods
- **D.** Report of chronic fatigue

🎨 *Suggested illustration:* a limp blood-pressure cuff reading 82/50 beside an IV bag of normal saline and a vial of hydrocortisone.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Hyperpigmentation is a classic *chronic* Addison's finding, not an emergency.
- **B — Correct.** In **Addisonian crisis**, the lack of cortisol and aldosterone causes **profound hypotension and vascular collapse.** The BP of 82/50 is life-threatening and needs immediate IV fluids and IV hydrocortisone.
- **C — Incorrect.** Salt craving reflects aldosterone deficiency but is not the acute priority.
- **D — Incorrect.** Fatigue is expected in Addison's and is not urgent.

📝 **NCLEX Tip. Addison's = "everything low"** (low cortisol, low BP, low sodium, low glucose, high potassium). **Cushing's = "everything high."** Crisis = treat the **shock** first.

💡 **Clinical Pearl / Memory Trick.** "**Add**ison's = you have to **add** the steroids." Priorities in crisis: **fluids, hydrocortisone, and correct the electrolytes.**

Question 15 — Diabetic ketoacidosis

Category: Adult Health / Endocrine · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client with type 1 diabetes is admitted with diabetic ketoacidosis (DKA): glucose 560 mg/dL, pH 7.20, and potassium 5.8 mEq/L. An IV insulin infusion and IV fluids are started.

As the insulin begins to work, which electrolyte change must the nurse anticipate and monitor closely?

- **A.** Rising sodium
- **B.** Falling potassium
- **C.** Rising calcium
- **D.** Falling glucose only, with no electrolyte concern

🎨 *Suggested illustration:* an arrow showing potassium moving from the bloodstream into a cell as insulin acts, with a cardiac monitor in the background.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Sodium shifts occur but are not the priority monitoring concern as insulin acts.
- **B — Correct.** Insulin drives **potassium back into the cells**, so a serum potassium that started high can **fall rapidly** into a dangerous low range. The nurse monitors potassium closely and anticipates replacement.
- **C — Incorrect.** Calcium is not the key electrolyte in DKA management.
- **D — Incorrect.** Glucose does fall, but ignoring potassium could be fatal — hypokalemia causes lethal dysrhythmias.

📝 **NCLEX Tip.** In DKA, **insulin lowers glucose *and* potassium.** The test loves the moment when initial hyperkalemia flips to hypokalemia. Never give IV insulin if potassium is already low without replacing it.

💡 **Clinical Pearl / Memory Trick. "Insulin pushes K into the cell."** Follow the sequence: **fluids first, then insulin, and watch the potassium the whole way down.**

Question 16 — Ischemic stroke and thrombolytics

Category: Adult Health / Neuro · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A client arrives in the emergency department with sudden left-sided weakness and slurred speech that began 90 minutes ago. A CT scan rules out hemorrhage, and the provider considers alteplase (tPA).

Which piece of information is most important for the nurse to verify before tPA is given?

- **A.** The client's usual blood pressure at home
- **B.** The exact time of symptom onset and any recent bleeding or surgery
- **C.** Whether the client has eaten in the last 8 hours
- **D.** The client's preferred language

🎨 *Suggested illustration:* a clock showing symptom onset with a shrinking "treatment window," beside a checklist that includes "recent surgery? recent bleeding?"

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Current BP matters for tPA parameters, but the *usual home* BP is not the critical screening item.
- **B — Correct.** tPA is time-limited (a narrow window from **symptom onset**) and is contraindicated with recent bleeding, surgery, or hemorrhagic stroke. Confirming **onset time** and **bleeding risk** is essential to give it safely.
- **C — Incorrect.** Fasting status is not the key thrombolytic screening question.
- **D — Incorrect.** Preferred language matters for communication but is not the safety priority before a clot-buster.

📝 **NCLEX Tip.** For thrombolytics, think **"time and bleeding."** If you don't know the *exact* onset time, or there's recent bleeding/surgery, tPA is unsafe.

💡 **Clinical Pearl / Memory Trick.** Stroke mantra: **"Time is brain."** Act **FAST** — **F**ace droop, **A**rm drift, **S**peech slurred, **T**ime to call 911.

Question 17 — Hemodialysis access care

Category: Adult Health / Renal · **Client Need:** Reduction of Risk Potential · **Difficulty:** Moderate

Scenario. A client has a newly matured arteriovenous (AV) fistula in the left arm for hemodialysis.

Which nursing action protects the fistula?

- **A.** Take blood pressures and draw blood from the left arm for convenience.
- **B.** Assess for a thrill (buzz) and bruit, and avoid constriction of the left arm.
- **C.** Keep a tight blood-pressure cuff inflated on the left arm to reduce bleeding.
- **D.** Apply a heating pad continuously over the fistula site.

🎨 *Suggested illustration:* a nurse's fingertips resting over a forearm fistula feeling a "thrill," with a "no BP / no blood draws" symbol over that arm.

✅ **Correct answer: B**

Rationale
- **A — Incorrect. No BP cuffs, blood draws, or IVs** in the fistula arm — these can clot or damage the access.
- **B — Correct.** The nurse confirms patency by **palpating the thrill** and **auscultating the bruit** each shift, and protects the arm from anything that compresses it (tight sleeves, cuffs, sleeping on it).
- **C — Incorrect.** A constantly inflated cuff would occlude the fistula and cause it to clot.
- **D — Incorrect.** Continuous heat is not standard fistula care and risks burns/vasodilation problems.

📝 **NCLEX Tip.** Fistula assessment = **"feel the thrill, hear the bruit."** Their absence means the access may have **clotted** — report immediately.

💡 **Clinical Pearl / Memory Trick.** Protect the access arm: **"No BP, no sticks, no squeeze."** A palpable **thrill** and audible **bruit** mean it's flowing.

Question 18 — Hepatic encephalopathy

Category: Adult Health / GI · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A client with cirrhosis is admitted with confusion and asterixis (a flapping hand tremor). The provider prescribes lactulose.

Which outcome tells the nurse the lactulose is working as intended?

- **A.** The client has two to three soft stools per day and improving mental status.
- **B.** The client reports relief of abdominal pain.
- **C.** The client's blood pressure decreases.
- **D.** The client has no bowel movements for 24 hours.

🎨 *Suggested illustration:* a lab trend showing ammonia falling next to a client who looks more alert, with a small stool-count tally of 2–3/day.

✅ **Correct answer: A**

Rationale
- **A — Correct.** Lactulose treats **hepatic encephalopathy** by trapping ammonia in the gut and expelling it in stool. The therapeutic goal is **2–3 soft stools daily**, which lowers ammonia and **improves mental status.**
- **B — Incorrect.** Lactulose is not an analgesic; pain relief is not its purpose or measure of effect.
- **C — Incorrect.** Lactulose does not act primarily on blood pressure.
- **D — Incorrect.** No bowel movements means the drug is **not** clearing ammonia — the opposite of the goal (and a sign the dose is too low).

📝 **NCLEX Tip.** For lactulose, **more stools is the goal, not a side effect.** Titrate to 2–3 soft stools/day. Rising ammonia → worsening confusion.

💡 **Clinical Pearl / Memory Trick.** "**Lactulose loses the ammonia** — out the back door." Falling ammonia + clearer thinking = success.

Question 19 — Acute pancreatitis

Category: Adult Health / GI · **Client Need:** Basic Care and Comfort · **Difficulty:** Moderate

Scenario. A client is admitted with acute pancreatitis and severe epigastric pain radiating to the back.

Which position and intervention will best promote comfort?

- **A.** Supine and flat with the legs extended
- **B.** Side-lying with knees drawn up toward the chest (fetal position)
- **C.** High-Fowler's with a large meal to settle the stomach
- **D.** Prone with the head turned to the side

🎨 *Suggested illustration:* a client curled in a fetal side-lying position holding the abdomen, with an "NPO" sign on the bedside table.

✅ **Correct answer: B**

Rationale
- A — Incorrect. Lying flat with legs extended stretches the abdomen and typically worsens pancreatic pain.
- B — Correct. A **side-lying, knees-to-chest (fetal)** position reduces tension on the inflamed pancreas and eases pain. The client is also kept **NPO** to rest the gland.
- C — Incorrect. Eating stimulates pancreatic enzyme secretion and intensifies pain — clients are NPO, not fed.
- D — Incorrect. Prone positioning does not relieve pancreatic pain and is impractical.

📝 **NCLEX Tip.** Pancreatitis care = **rest the pancreas**: NPO, IV fluids, pain control, and a position that *unstretches* the abdomen (fetal/leaning forward).

💡 **Clinical Pearl / Memory Trick.** "**Curl up to calm the pancreas.**" Remember the enzymes: **amylase and lipase rise** (lipase is more specific).

Question 20 — Autonomic dysreflexia

Category: Adult Health / Neuro · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client with a spinal cord injury at the T4 level suddenly develops a pounding headache, blood pressure 200/110 mm Hg, flushing above the injury, and sweating.

What should the nurse do first?

- **A.** Lay the client flat and elevate the legs.
- **B.** Raise the head of the bed to a sitting position and look for the trigger (often a full bladder).
- **C.** Administer an as-needed sedative and dim the lights.
- **D.** Notify the provider and wait for orders before acting.

🎨 *Suggested illustration:* a client sitting upright with a red flushed face and a bulging bladder icon, blood pressure cuff reading 200/110.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Lying flat and raising the legs would *increase* an already dangerously high blood pressure.
- **B — Correct.** Autonomic dysreflexia is a hypertensive emergency. The nurse immediately **sits the client up** (to lower BP by orthostasis) and **removes the trigger** — most commonly a **distended bladder** (kinked catheter), impacted stool, or tight clothing.
- **C — Incorrect.** Sedation and dim lights do not address the surging blood pressure or its cause.
- **D — Incorrect.** This is time-critical; the nurse acts first (sit up, find the trigger) *and* notifies the provider — not waits idly.

📝 **NCLEX Tip.** Autonomic dysreflexia = **the one time you raise the head and lower the legs for high BP.** Then hunt the trigger: **bladder → bowel → skin.**

💡 **Clinical Pearl / Memory Trick. "Sit them up, find the cause."** The culprit is usually a **full bladder** — check the catheter first. Untreated, it can cause a stroke or seizure.

Section 3 · Maternal, Newborn & Pediatrics

Question 21 — Preeclampsia warning sign

Category: Maternity · **Client Need:** Health Promotion and Maintenance · **Difficulty:** Moderate

Scenario. A client at 37 weeks' gestation with preeclampsia is being monitored on the antepartum unit.

Which new report should the nurse recognize as a sign of worsening disease and report immediately?

- **A.** Mild ankle edema at the end of the day
- **B.** Epigastric or right-upper-quadrant pain
- **C.** Occasional fetal hiccups
- **D.** Braxton Hicks contractions that stop with rest

🎨 *Suggested illustration:* a pregnant client pressing a hand to the upper-right abdomen with a small "liver" highlight, and a warning triangle over the area.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Dependent ankle edema is common in late pregnancy and, by itself, is not an emergency.
- **B — Correct. Epigastric or RUQ pain** signals liver involvement (hepatic distention, possible HELLP syndrome) and often precedes a **seizure (eclampsia).** It is an urgent finding.
- **C — Incorrect.** Fetal hiccups are a normal, reassuring finding.
- **D — Incorrect.** Braxton Hicks contractions that resolve with rest are normal and not a sign of worsening preeclampsia.

📝 **NCLEX Tip.** The preeclampsia danger trio to report: **severe headache, visual changes, and epigastric/RUQ pain.** These warn of impending eclampsia.

💡 **Clinical Pearl / Memory Trick.** "**Head, eyes, and belly = seizure on the way.**" Epigastric pain in preeclampsia is a **liver alarm**, not indigestion.

Question 22 — Fetal heart rate: late decelerations

Category: Maternity / Intrapartum · **Client Need:** Reduction of Risk Potential · **Difficulty:** Hard

Scenario. A laboring client receiving IV oxytocin shows **late decelerations** on the fetal monitor — the fetal heart rate drops after the peak of each contraction and returns to baseline after the contraction ends.

What is the nurse's first action?

- **A.** Increase the oxytocin infusion to strengthen contractions.
- **B.** Stop the oxytocin, reposition the client to her side, and give oxygen.
- **C.** Encourage the client to hold her breath and push with each contraction.
- **D.** Document the reassuring tracing and continue routine care.

🎨 *Suggested illustration:* a fetal monitor strip showing the heart-rate dip mirroring and lagging behind each contraction, with an arrow to a side-lying laboring client on oxygen.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Increasing oxytocin worsens uteroplacental insufficiency and the decelerations — exactly the wrong move.
- **B — Correct.** Late decelerations indicate **uteroplacental insufficiency** (the fetus isn't getting enough oxygen). Intrauterine resuscitation is priority: **stop the oxytocin, turn the client on her side, give oxygen, and increase IV fluids.**
- **C — Incorrect.** Pushing does not correct the underlying oxygen problem.
- **D — Incorrect.** Late decelerations are **non-reassuring**, not normal — they require action.

📝 **NCLEX Tip.** Memorize the pattern letters: **VEAL CHOP. L**ate decels → **P**lacental insufficiency → act. **V**ariable → **C**ord compression → reposition.

💡 **Clinical Pearl / Memory Trick.** Intrauterine resuscitation = **"Turn, O_2, IV, stop the Pit."** Late = **late is bad** (placental problem); early decels mirror the contraction and are usually benign (head compression).

Question 23 — Postpartum hemorrhage

Category: Maternity / Postpartum · **Client Need:** Reduction of Risk Potential · **Difficulty:** Moderate

Scenario. One hour after a vaginal birth, the nurse finds the client's uterine fundus **boggy (soft)** and displaced above and to the right of the umbilicus, with a large amount of vaginal bleeding.

What should the nurse do first?

- **A.** Massage the fundus while supporting the lower uterine segment.
- **B.** Administer a prescribed analgesic for afterbirth pains.
- **C.** Increase the rate of the IV fluids and recheck in 30 minutes.
- **D.** Place the client in reverse Trendelenburg position.

🎨 *Suggested illustration:* a nurse's two hands on the abdomen — one massaging a soft fundus, one cupping just above the pubic bone — with a full-bladder icon pushing the uterus off-center.

✅ **Correct answer: A**

Rationale
- **A — Correct.** A **boggy fundus** means the uterus isn't contracting (uterine atony), the leading cause of early postpartum hemorrhage. **Fundal massage** is the immediate action to firm the uterus and control bleeding. (A displaced fundus also hints at a full bladder, which should be emptied next.)
- **B — Incorrect.** Pain control does not address the active hemorrhage.
- **C — Incorrect.** Fluids support volume but do not stop the bleeding; waiting 30 minutes is unsafe.
- **D — Incorrect.** Positioning does not treat uterine atony.

📝 **NCLEX Tip.** Boggy uterus = **massage first.** A uterus pushed up and to the **right** is a classic clue for a **full bladder** — have the client void or catheterize after massaging.

💡 **Clinical Pearl / Memory Trick.** "**Firm it up.**" If massage alone doesn't work, expect uterotonics (**oxytocin, methylergonovine, carboprost**) — but **hold methylergonovine if the client is hypertensive.**

Question 24 — Normal newborn assessment

Category: Newborn · **Client Need:** Health Promotion and Maintenance · **Difficulty:** Easy

Scenario. A nurse assesses a 2-hour-old term newborn.

Which finding is normal and requires no further action?

- **A.** Bluish discoloration of the hands and feet (acrocyanosis)
- **B.** A respiratory rate of 22/min
- **C.** Central cyanosis of the lips and trunk
- **D.** A blood glucose of 30 mg/dL

🎨 *Suggested illustration:* a swaddled newborn with slightly bluish hands and feet but a pink face and trunk, a small "normal" checkmark near the extremities.

✅ **Correct answer: A**

Rationale
- **A — Correct. Acrocyanosis** (blue hands and feet with a pink trunk) is a normal newborn finding in the first 24–48 hours due to immature peripheral circulation.
- **B — Incorrect.** Normal newborn respirations are **30–60/min**; 22 is too low and needs evaluation.
- **C — Incorrect. Central** cyanosis (lips, tongue, trunk) is never normal and signals hypoxia — an emergency.
- **D — Incorrect.** Newborn glucose below **40 mg/dL** suggests hypoglycemia and requires intervention (feeding, rechecking).

📝 **NCLEX Tip.** Distinguish **acrocyanosis (normal, peripheral)** from **central cyanosis (abnormal, trunk/lips).** Only the peripheral kind is expected.

💡 **Clinical Pearl / Memory Trick.** Newborn normals to bank: **HR 110–160, RR 30–60, temp ~36.5–37.5°C, glucose ≥ 40.** Blue *ends* okay, blue *center* not.

Question 25 — Antenatal corticosteroids

Category: Maternity / Pharmacology · **Client Need:** Pharmacological Therapies · **Difficulty:** Moderate

Scenario. A client in preterm labor at 30 weeks receives IM betamethasone.
The client asks why it was given.

Which response by the nurse is accurate?

- **A.** "It stops your contractions so labor won't progress."
- **B.** "It helps your baby's lungs mature more quickly in case of early birth."
- **C.** "It treats an infection that may have triggered your labor."
- **D.** "It raises your blood pressure to protect the placenta."

🎨 *Suggested illustration:* a cross-section of developing fetal lungs with tiny alveoli filling with surfactant, a syringe labeled "betamethasone" beside it.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Betamethasone does not stop contractions — that's the role of tocolytics (e.g., magnesium sulfate, nifedipine).
- **B — Correct.** Antenatal corticosteroids like **betamethasone accelerate fetal lung maturity** (surfactant production), reducing respiratory distress syndrome if the baby is born preterm.
- **C — Incorrect.** It is not an antibiotic and does not treat infection.
- **D — Incorrect.** Its purpose is fetal lung maturation, not raising maternal blood pressure.

📝 **NCLEX Tip.** Separate the two preterm-labor drugs: **tocolytics buy time** (stop contractions); **corticosteroids mature the lungs.** Betamethasone = lungs.

💡 **Clinical Pearl / Memory Trick.** "**Beta-methasone = better breathing.**" Given to mom, works on the **baby's lungs.**

Question 26 — Infant safe sleep

Category: Pediatrics / Health Promotion · **Client Need:** Health Promotion and Maintenance · **Difficulty:** Easy

Scenario. A nurse is teaching new parents about reducing the risk of sudden infant death syndrome (SIDS).

Which instruction should the nurse include?

- **A.** "Place your baby on the stomach to sleep to prevent choking."
- **B.** "Put your baby to sleep on the back on a firm, flat surface with no soft bedding."
- **C.** "Use a soft pillow and plush blankets to keep the baby comfortable."
- **D.** "Let the baby sleep in your bed so you can watch them closely."

🎨 *Suggested illustration:* an infant lying on its back in an empty crib with a firm mattress and no pillows or toys, a green "safe sleep" checkmark above.

✅ **Correct answer: B**

Rationale
- **A — Incorrect. Stomach (prone) sleeping raises SIDS risk** — the outdated choking fear is not supported; healthy infants protect their airway on their backs.
- **B — Correct.** "**Back to sleep**" on a **firm, flat, bare surface** — no pillows, bumpers, or loose blankets — is the evidence-based standard to reduce SIDS.
- **C — Incorrect.** Soft pillows and plush bedding are suffocation hazards.
- **D — Incorrect.** Bed-sharing increases the risk of suffocation and SIDS; room-sharing (separate sleep surface) is what's recommended.

📝 **NCLEX Tip.** Safe sleep = **Back, Bare, Alone (own surface).** Any answer adding pillows, blankets, or prone positioning is wrong.

💡 **Clinical Pearl / Memory Trick.** "**Back to sleep, tummy to play.**" The crib should be **boring** — nothing in it but the baby.

Question 27 — Suspected epiglottitis

Category: Pediatrics / Respiratory · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A 4-year-old arrives drooling, leaning forward in a tripod position, with a high fever, muffled voice, and refusal to swallow. Epiglottitis is suspected.

Which action should the nurse avoid?

- **A.** Keeping the child calm and in a position of comfort on the parent's lap
- **B.** Examining the throat with a tongue depressor to visualize the epiglottis
- **C.** Preparing emergency airway equipment at the bedside
- **D.** Notifying the provider and staying with the child

🎨 *Suggested illustration:* a child sitting upright in tripod position drooling, with a red "do not" symbol over a tongue depressor near the mouth.

✅ **Correct answer: B**

Rationale
- **A — Incorrect (this is appropriate).** Keeping the child calm and upright prevents further airway spasm — a correct action, so it is not what the nurse avoids.
- **B — Correct (this is what to AVOID). Never inspect the throat or use a tongue depressor** with suspected epiglottitis — it can trigger laryngospasm and complete airway obstruction. Visualization is done only where an emergency airway can be secured.
- **C — Incorrect (this is appropriate).** Having airway/intubation equipment ready is essential.
- **D — Incorrect (this is appropriate).** Staying with the child and notifying the provider is correct.

📝 **NCLEX Tip.** This is a **negative-style** item ("which should you AVOID"). Read the stem twice — the *right answer is the wrong action.* For epiglottitis: **hands off the throat.**

💡 **Clinical Pearl / Memory Trick.** Epiglottitis "**tripod, drool, and muffled**" = airway emergency. Remember the **4 D's: Drooling, Dysphagia, Dysphonia, Distress.** Don't touch the throat.

Question 28 — Pediatric dehydration

Category: Pediatrics / Fluid & Electrolyte · **Client Need:** Reduction of Risk Potential · **Difficulty:** Moderate

Scenario. A nurse is monitoring an infant admitted with several days of vomiting and diarrhea.

Which finding is the most reliable indicator of the infant's fluid status?

- **A.** Skin color
- **B.** Daily weight
- **C.** Reported number of wet diapers by phone
- **D.** Presence of tears when crying

🎨 *Suggested illustration:* an infant scale with a trend line dropping day over day, next to a diaper with a small "output" note.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Skin color is nonspecific and affected by many factors.
- **B — Correct. Daily weight** measured on the same scale is the **most accurate, objective indicator of fluid balance** in infants and children — 1 kg ≈ 1 L of fluid. Rapid weight loss quantifies dehydration.
- **C — Incorrect.** Diaper counts help but are subjective and less precise than measured weight.
- **D — Incorrect.** Absence of tears and a sunken fontanel are *signs* of dehydration but don't measure it as reliably as weight.

📝 **NCLEX Tip.** When a question asks for the **best/most reliable** measure of fluid status, **daily weight** usually wins over intake/output estimates.

💡 **Clinical Pearl / Memory Trick.** "**The scale tells the tale.**" Other infant dehydration clues: **sunken fontanel, no tears, dry mucous membranes, and decreased urine output (< 1 mL/kg/hr).**

Question 29 — Cystic fibrosis nutrition

Category: Pediatrics / GI · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A child with cystic fibrosis is prescribed pancreatic enzyme replacement.

Which instruction should the nurse give the caregivers?

- **A.** "Give the enzymes only at bedtime on an empty stomach."
- **B.** "Give the enzymes with every meal and snack."
- **C.** "Restrict the child's fat and calorie intake."
- **D.** "Skip the enzymes on days the child feels well."

🎨 *Suggested illustration:* a plate of food beside an enzyme capsule with a clock showing "with meals," and a high-calorie snack in the background.

✅ **Correct answer: B**

Rationale
- A — Incorrect. Enzymes must be taken **with food** to help digest it, not on an empty stomach.
- B — Correct. In cystic fibrosis, thick secretions block pancreatic enzymes, causing malabsorption. **Pancreatic enzymes are given with every meal and snack** so nutrients (especially fats and fat-soluble vitamins) can be absorbed.
- C — Incorrect. These children need a **high-calorie, high-protein** diet — fat is not restricted; it's supported with enzymes.
- D — Incorrect. Enzymes are needed with all food, every day — not skipped.

📝 **NCLEX Tip.** Rule of thumb for replacement enzymes: **"No food, no enzyme; food, then enzyme."** CF kids need **more** calories, not fewer.

💡 **Clinical Pearl / Memory Trick.** CF = **"salty, sticky, and hungry."** Salty sweat (diagnostic sweat chloride), sticky mucus, and a big appetite need to **eat more + take enzymes + add fat-soluble vitamins A, D, E, K.**

Question 30 — Tetralogy of Fallot "tet spell"

Category: Pediatrics / Cardiac · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A toddler with tetralogy of Fallot suddenly becomes acutely cyanotic and irritable during crying (a hypercyanotic "tet" spell).

What should the nurse do first?

- **A.** Lay the child flat and extend the legs.
- **B.** Place the child in a knee-to-chest position.
- **C.** Encourage vigorous physical activity to improve circulation.
- **D.** Withhold oxygen to avoid drying the airways.

🎨 *Suggested illustration:* a caregiver holding a toddler with knees tucked up to the chest, a calm posture, oxygen flowing, cyanosis easing.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Lying flat with legs extended reduces systemic resistance and worsens the right-to-left shunt and cyanosis.
- **B — Correct.** The **knee-to-chest** position **increases systemic vascular resistance**, which reduces the right-to-left shunt and directs more blood to the lungs — relieving the cyanotic spell. Oxygen and calming the child follow.
- **C — Incorrect.** Activity increases oxygen demand and deepens cyanosis.
- **D — Incorrect.** Oxygen is *given*, not withheld, during a hypercyanotic spell.

📝 **NCLEX Tip.** For a tet spell, the magic position is **knee-to-chest** (older children may squat). Both increase systemic resistance and ease the shunt.

💡 **Clinical Pearl / Memory Trick.** "**Knees to chest gives the lungs their best.**" Squatting/knee-chest is the toddler's own instinct to abort a tet spell.

Section 4 · Mental Health & Fundamentals

Question 31 — Therapeutic communication

Category: Mental Health · **Client Need:** Psychosocial Integrity · **Difficulty:** Moderate

Scenario. A client newly diagnosed with cancer says, "I don't even know how to tell my kids. I feel like I've already let them down."

Which response by the nurse is most therapeutic?

- **A.** "Don't worry — plenty of people beat cancer these days."
- **B.** "It sounds like you're feeling overwhelmed about facing your children."
- **C.** "Why would you feel like you've let them down? You didn't cause this."
- **D.** "You should focus on staying positive for your family's sake."

🎨 *Suggested illustration:* a nurse seated at eye level, leaning in, one hand open toward a client who looks down — an unhurried, attentive posture.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** False reassurance ("plenty of people beat cancer") dismisses the client's feelings and shuts down communication.
- **B — Correct. Reflecting the client's feeling** ("it sounds like you're feeling overwhelmed") shows empathy, validates the emotion, and invites the client to say more — the heart of therapeutic communication.
- **C — Incorrect.** "Why" questions can sound challenging and put the client on the defensive.
- **D — Incorrect.** Telling the client to "stay positive" minimizes real distress and imposes the nurse's agenda.

📝 **NCLEX Tip.** Therapeutic answers usually **reflect feelings, stay open, and keep the focus on the client.** Avoid false reassurance, "why" questions, giving advice, and changing the subject.

💡 **Clinical Pearl / Memory Trick.** When two answers both look kind, pick the one that **names the feeling and hands the conversation back to the client.** Silence and reflection beat cheerleading.

Question 32 — Suicide risk priority

Category: Mental Health · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Hard

Scenario. During an assessment, a client with depression states, "I've been saving up my pills, and soon it won't matter anymore."

What is the nurse's priority action?

- **A.** Encourage the client to focus on reasons for living.
- **B.** Ask the client directly whether they have a plan to harm themselves and ensure safety.
- **C.** Document the statement and continue the scheduled assessment.
- **D.** Reassure the client that things will improve with medication.

🎨 *Suggested illustration:* a nurse making calm, direct eye contact asking a clear question, with a "safety first" band across the bottom.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Reasons for living may come later, but they do not address the immediate danger.
- **B — Correct.** The statement suggests a **plan and means** (saved pills). The nurse must **ask directly about suicidal intent and plan** and **institute safety measures** immediately (do not leave the client alone, remove means). Asking directly does not "plant the idea."
- **C — Incorrect.** Documenting without acting leaves the client in danger.
- **D — Incorrect.** Reassurance ignores an active safety threat.

📝 **NCLEX Tip.** Safety is always the priority in suicide items. A client with a **plan and the means** is high-risk and needs **direct questioning and immediate protection.**

💡 **Clinical Pearl / Memory Trick.** "**Ask the question, remove the means, never leave alone.**" The more specific the **plan** and the more available the **means**, the higher the risk.

Question 33 — Alcohol withdrawal

Category: Mental Health / Physiological · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A client admitted 24 hours ago after a fall reports the last alcoholic drink was "yesterday morning." The nurse notes tremors, diaphoresis, heart rate 118/min, blood pressure 168/96, and rising anxiety.

Which is the nurse's priority concern?

- **A.** These findings are expected and require no action.
- **B.** The client is at risk for worsening withdrawal, including seizures and delirium tremens.
- **C.** The client is likely experiencing an allergic reaction.
- **D.** The client is showing signs of opioid intoxication.

🎨 *Suggested illustration:* a timeline of hours-since-last-drink with escalating symptoms (tremor → tachycardia → seizure risk), a vital-signs monitor trending up.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Escalating autonomic signs are **not** benign — untreated, withdrawal can progress to seizures and **delirium tremens (DTs)**, which can be fatal.
- **B — Correct.** Tremor, diaphoresis, tachycardia, hypertension, and anxiety 1–2 days after the last drink signal **alcohol withdrawal.** The nurse anticipates **benzodiazepines** (per protocol/CIWA scoring), thiamine, a calm environment, and seizure precautions.
- **C — Incorrect.** There is no allergen described; the picture is autonomic hyperactivity, not anaphylaxis.
- **D — Incorrect.** Opioid intoxication causes **sedation and slowed breathing** — the opposite of this hyperadrenergic state.

📝 **NCLEX Tip.** Alcohol withdrawal = **hyperactivity** (up HR, up BP, tremor, agitation). Peak DT risk is **48–72 hours** after the last drink. Benzodiazepines are the mainstay.

💡 **Clinical Pearl / Memory Trick.** "**Withdrawal winds you up.**" Give **thiamine before glucose** to prevent Wernicke's encephalopathy in heavy drinkers.

Question 34 — Lithium and fluid balance teaching

Category: Mental Health / Pharmacology · **Client Need:** Health Promotion and Maintenance · **Difficulty:** Easy

Scenario. A nurse is teaching a client starting lithium about self-care at home.

Which client statement indicates a need for further teaching?

- **A.** "I'll drink plenty of water and keep my salt intake steady."
- **B.** "I'll get my blood levels checked as scheduled."
- **C.** "I'll start a strict low-sodium diet to be healthier."
- **D.** "I'll call my provider if I have vomiting or diarrhea."

🎨 *Suggested illustration:* a water bottle and a steady salt shaker with a green check, and a "low-sodium diet" label crossed out in red.

✅ **Correct answer: C**

Rationale
- **A — Incorrect (correct understanding).** Adequate fluids and **consistent** sodium keep lithium levels stable — good teaching.
- **B — Incorrect (correct understanding).** Regular level monitoring is essential with lithium's narrow therapeutic range.
- **C — Correct (this statement needs correction).** A **low-sodium diet is dangerous** with lithium — low sodium makes the kidneys retain lithium, raising it toward toxicity. This shows a knowledge gap.
- **D — Incorrect (correct understanding).** Vomiting and diarrhea cause fluid/sodium loss and toxicity risk, so reporting them is correct.

📝 **NCLEX Tip.** Watch for **negative-polarity** stems ("need for further teaching," "requires follow-up") — you're hunting the **wrong** statement.

💡 **Clinical Pearl / Memory Trick.** Lithium loves **steady salt and steady water.** Sudden **low sodium or dehydration = rising lithium = toxicity.**

Question 35 — Blood pressure measurement error

Category: Fundamentals · **Client Need:** Reduction of Risk Potential · **Difficulty:** Easy

Scenario. A nurse obtains an unexpectedly high blood pressure reading on a client.

Which factor could falsely elevate the reading?

- **A.** Using a cuff that is too small (narrow) for the client's arm
- **B.** Positioning the arm at the level of the heart
- **C.** Allowing the client to rest quietly for five minutes first
- **D.** Using a cuff that covers about 80% of the upper arm circumference

🎨 *Suggested illustration:* a too-small cuff straining around a large arm with an exaggerated high number on the monitor, next to a correctly sized cuff reading normal.

✅ **Correct answer: A**

Rationale
- **A — Correct.** A cuff that is **too small/narrow** for the arm gives a **falsely high** reading. (A cuff too large reads falsely low.)
- **B — Incorrect.** Arm at heart level is the **correct** technique and yields an accurate reading.
- **C — Incorrect.** Resting quietly first prevents falsely high readings — good technique.
- **D — Incorrect.** A properly sized cuff (bladder covering ~80% of the arm) is correct and improves accuracy.

📝 **NCLEX Tip.** Cuff-size errors: **too small → too high; too big → too low.** Also, an arm **below** heart level reads high; **above** heart level reads low.

💡 **Clinical Pearl / Memory Trick.** "**Small cuff, big number.**" When a value surprises you, **recheck technique before treating the number.**

Question 36 — Sterile field / surgical asepsis

Category: Fundamentals / Infection Control · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Moderate

Scenario. A nurse is setting up a sterile field for a bedside procedure.

Which action breaks sterile technique and requires the nurse to start over?

- **A.** Holding sterile items above waist level and within view
- **B.** Considering the outer one-inch border of the sterile drape contaminated
- **C.** Reaching across the sterile field to place an item on the far side
- **D.** Opening the outermost flap of a sterile package away from the body first

🎨 *Suggested illustration:* a gloved hand reaching directly over an open sterile tray with a red "contaminated" arc where the arm crosses it.

✅ **Correct answer: C**

Rationale
- A — Incorrect (correct technique). Keeping sterile items **above the waist and in sight** maintains sterility.
- B — Incorrect (correct technique). The **outer 1-inch border** of a sterile field is treated as **contaminated** — standard practice.
- C — Correct (this breaks sterility). Reaching across a sterile field contaminates it — the unsterile arm passes over sterile items. The nurse must set up so items are reached from the side, not over the top.
- D — Incorrect (correct technique). Opening the **first flap away** from the body prevents reaching over the sterile contents.

📝 **NCLEX Tip.** Sterile rules to bank: **keep it in sight, above the waist; 1-inch border is dirty; never turn your back on it; never reach across it; wet = contaminated.**

💡 **Clinical Pearl / Memory Trick.** "**Below the waist or out of sight = it isn't sterile — not quite.**" When in doubt, throw it out and restart.

Question 37 — Nasogastric tube placement verification

Category: Fundamentals · **Client Need:** Reduction of Risk Potential · **Difficulty:** Moderate

Scenario. A nurse prepares to give medication through a newly inserted nasogastric (NG) tube.

Which method is the most reliable for verifying correct tube placement?

- **A.** Auscultating air injected into the tube over the stomach
- **B.** Checking that the client is not coughing
- **C.** Reviewing the X-ray obtained after insertion
- **D.** Confirming the external tube length looks about right

🎨 *Suggested illustration:* a chest/abdomen X-ray showing an NG tube tip below the diaphragm in the stomach, marked with a confirming checkmark.

✅ **Correct answer: C**

Rationale
- A — Incorrect. The **air/auscultation ("whoosh") test is unreliable** — sounds can transmit even if the tube is in the lung. It's no longer recommended as confirmation.
- B — Incorrect. Absence of coughing does not confirm gastric placement; a tube can be misplaced without coughing.
- C — Correct. Radiographic (X-ray) confirmation is the **gold standard** for verifying placement after initial insertion, before using the tube.
- D — Incorrect. External length is a rough check for migration, not proof of correct initial placement.

📝 **NCLEX Tip.** For **initial** NG placement, the answer is **X-ray.** Ongoing checks may use **pH of aspirate** and measuring the external length — but never the air-auscultation method as confirmation.

💡 **Clinical Pearl / Memory Trick.** "**When in doubt, X-ray it out.**" Gastric aspirate is usually **acidic (pH ≤ 5)**; respiratory/intestinal fluid is more alkaline.

Question 38 — Fall risk / restraint alternatives

Category: Fundamentals / Safety · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Easy

Scenario. A confused older adult is at high risk for falls. The nurse plans care to keep the client safe.

Which intervention should the nurse implement first?

- **A.** Apply a vest restraint to keep the client in bed.
- **B.** Place the bed in the lowest position with the call light in reach and initiate frequent rounding.
- **C.** Raise all four side rails to prevent the client from getting up.
- **D.** Keep the room dark and quiet at all times to reduce stimulation.

🎨 *Suggested illustration:* a low bed with a bed-alarm pad, call light clipped within reach, and a nurse checking in — no restraints in sight.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Restraints are a **last resort**, require an order, and can increase injury — not a first-line fall strategy.
- **B — Correct.** The **least restrictive** measures come first: **low bed, call light within reach, nonslip footwear, a clear path, bed/chair alarms, and frequent (hourly) rounding.**
- **C — Incorrect. Four raised side rails are considered a restraint** and can cause serious injury if the client climbs over.
- **D — Incorrect.** A completely dark room can increase confusion and fall risk; some soft lighting aids orientation.

📝 **NCLEX Tip.** Always choose the **least restrictive** safe option first. Restraints and four side rails are **restraints** — near the bottom of the list, never the first answer.

💡 **Clinical Pearl / Memory Trick. "Least restrictive, first and best."** Restraints need an **order, a time limit, and regular checks** — never PRN, never for staff convenience.

Question 39 — Hand hygiene and C. difficile

Category: Fundamentals / Infection Control · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Moderate

Scenario. A nurse cares for a client with *Clostridioides difficile* (C. diff) infection.

Which infection-control action is correct?

- **A.** Use alcohol-based hand sanitizer after removing gloves.
- **B.** Wash hands with soap and water, and use contact precautions with gown and gloves.
- **C.** Place the client on airborne precautions with an N95 respirator.
- **D.** No special precautions are needed if the client uses a private bathroom.

🎨 *Suggested illustration:* a nurse washing hands with soap and running water, a gown-and-gloves station outside a room marked "Contact Precautions."

✅ **Correct answer: B**

Rationale
- A — Incorrect. Alcohol sanitizer does not kill C. diff spores — soap and water friction is required to physically remove them.
- B — Correct. C. diff requires **contact precautions (gown and gloves)** and **handwashing with soap and water.** Dedicated equipment and bleach-based cleaning are also used.
- C — Incorrect. C. diff spreads by contact/fecal-oral, **not airborne** — an N95 is unnecessary.
- D — Incorrect. Precautions are absolutely required; a private bathroom does not replace contact isolation.

📝 **NCLEX Tip.** Remember the "spore rule": for **C. diff (and norovirus)**, use **soap and water**, not alcohol gel. Precaution type = **contact.**

💡 **Clinical Pearl / Memory Trick.** "**Spores need soap.**" Match the bug to the precaution: **Contact** (C. diff, MRSA), **Droplet** (flu, meningitis, pertussis), **Airborne** (**M**easles, **TB**, **V**aricella — "**My Chicken Hez TB**").

Question 40 — Delegation to unlicensed assistive personnel

Category: Fundamentals / Management · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Moderate

Scenario. A registered nurse (RN) is delegating tasks to unlicensed assistive personnel (UAP) on a busy med-surg unit.

Which task is appropriate for the RN to delegate to the UAP?

- **A.** Performing the admission assessment on a new client
- **B.** Recording the intake and output of a stable client
- **C.** Teaching a client how to use an incentive spirometer for the first time
- **D.** Adjusting the flow rate of a client's IV fluids

🎨 *Suggested illustration:* a task board sorting duties into "RN only" (assess, teach, evaluate, meds) and "UAP okay" (vitals, I&O, hygiene, ambulate).

✅ **Correct answer: B**

Rationale
- **A — Incorrect. Assessment** is an RN responsibility and cannot be delegated.
- **B — Correct.** Recording **intake and output** for a **stable** client is a routine, standardized task within the UAP's scope.
- **C — Incorrect. Teaching** (client education) is an RN function, especially first-time instruction.
- **D — Incorrect.** Titrating/adjusting IV fluids involves nursing judgment and is not delegated to a UAP.

📝 **NCLEX Tip.** UAPs handle **stable, routine, standard** tasks — vitals, hygiene, feeding, ambulating, I&O. The RN keeps the **"AT-E"**: **A**ssess, **T**each, **E**valuate (and nursing judgment/meds).

💡 **Clinical Pearl / Memory Trick.** "**If it takes a nurse's brain, it stays with the nurse.**" Delegate the **stable and predictable**, keep the **unstable and the teaching.**

Section 5 · Safety, Infection Control & Clinical Judgment

Question 41 — Prioritizing client care

Category: Clinical Judgment / Prioritization · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Hard

Scenario. At the start of the shift, a nurse receives report on four clients.

Which client should the nurse assess first?

- **A.** A client scheduled for discharge who is waiting for paperwork
- **B.** A client with new-onset shortness of breath and an oxygen saturation of 86%
- **C.** A client requesting pain medication for chronic back pain rated 5/10
- **D.** A postoperative client whose dressing has a small amount of dried drainage

🎨 *Suggested illustration:* four bedside cards ranked by urgency, the breathing client flagged red at the top with a pulse-ox reading 86%.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** A stable client awaiting discharge is the lowest priority.
- **B — Correct. Airway/Breathing problems come first.** New shortness of breath with an **SpO_2 of 86%** is an acute oxygenation emergency and takes priority over all the others.
- **C — Incorrect.** Chronic pain at 5/10 is important but not immediately life-threatening.
- **D — Incorrect.** A small amount of **dried** drainage is expected and stable.

📝 **NCLEX Tip.** Use **ABCs, then Maslow, then acute-over-chronic, then unstable-over-stable.** A breathing problem almost always outranks pain, discharge, or expected findings.

💡 **Clinical Pearl / Memory Trick.** "**Air goes first.**" Rank with **ABC** — then remember that **new/sudden and unstable** beats **chronic and expected** every time.

Question 42 — Sepsis interventions (Select All That Apply)

Category: Clinical Judgment · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client is suspected of having sepsis: temperature 39.2°C, heart rate 122/min, blood pressure 88/50 mm Hg, and rising lactate.

Which interventions should the nurse anticipate? Select all that apply.

- **A.** Obtain blood cultures before starting antibiotics.
- **B.** Administer broad-spectrum antibiotics promptly.
- **C.** Begin IV fluid resuscitation for hypotension.
- **D.** Withhold all fluids to avoid overload.
- **E.** Measure serum lactate and monitor urine output.

🎨 *Suggested illustration:* a "sepsis bundle" checklist — cultures, antibiotics, fluids, lactate, urine output — with the "withhold fluids" line struck through in red.

✅ **Correct answers: A, B, C, E**

Rationale
- **A — Correct. Cultures are drawn before antibiotics** so the causative organism can still be identified.
- **B — Correct. Early broad-spectrum antibiotics** (ideally within the first hour) are central to sepsis survival.
- **C — Correct. IV fluid resuscitation** treats the sepsis-induced hypotension and poor perfusion.
- **D — Incorrect.** Withholding fluids in a hypotensive septic client worsens perfusion and organ damage — the opposite of care.
- **E — Correct. Lactate** trends and **urine output** gauge perfusion and the response to treatment.

📝 **NCLEX Tip.** In SATA items, evaluate **each option as its own true/false.** One clearly harmful choice (like "withhold fluids") is almost always false.

💡 **Clinical Pearl / Memory Trick.** Sepsis "hour-1" bundle: **"Culture, Cover, Fluids, Lactate."** Draw cultures → give antibiotics → resuscitate with fluids → recheck lactate.

Question 43 — Hyperkalemia

Category: Clinical Judgment / Fluid & Electrolyte · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client with acute kidney injury has a serum potassium of 6.9 mEq/L. The cardiac monitor shows tall, peaked T waves.

Which action should the nurse take first?

- **A.** Encourage the client to eat a banana for energy.
- **B.** Notify the provider and prepare to treat the hyperkalemia while monitoring the ECG.
- **C.** Administer a potassium supplement as previously scheduled.
- **D.** Restrict the client's fluids and recheck the potassium in the morning.

🎨 *Suggested illustration:* an ECG strip with exaggerated tall, peaked T waves beside a lab value "K^+ 6.9" flagged red, and a cardiac monitor in view.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Bananas are **high in potassium** — exactly wrong for hyperkalemia.
- **B — Correct.** A potassium of 6.9 with **peaked T waves** is a cardiac emergency. The nurse **notifies the provider and prepares treatment** (e.g., IV calcium gluconate to protect the heart; insulin with dextrose and/or a beta-agonist to shift potassium into cells; and measures to remove it) while watching the ECG.
- **C — Incorrect.** Giving more potassium to a hyperkalemic client could be lethal — always reassess "scheduled" electrolytes against current labs.
- **D — Incorrect.** Waiting until morning with a life-threatening level and ECG changes is unsafe.

📝 **NCLEX Tip.** Match the ECG to the electrolyte: **hyperkalemia → tall, peaked T waves; hypokalemia → flat T waves and U waves.** Peaked T's + high K = act now.

💡 **Clinical Pearl / Memory Trick.** Emergency hyperkalemia order: **"Calcium protects, insulin + D50 shifts, and dialysis/kayexalate removes."** Calcium guards the heart first; it doesn't lower the potassium itself.

Question 44 — Disaster triage

Category: Safety / Management · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Moderate

Scenario. After a mass-casualty event, a charge nurse must free up medical-surgical beds quickly.

Which client is the most appropriate to discharge or transfer first to make room?

- **A.** A client 2 days post-op who is stable, tolerating a diet, and ambulating
- **B.** A client receiving IV vasopressors for unstable blood pressure
- **C.** A client with new chest pain and pending cardiac enzymes
- **D.** A client with a temperature of 39.5°C and a rising white blood cell count

🎨 *Suggested illustration:* a triage board moving one green-tagged stable client toward the exit while red-tagged unstable clients stay.

✅ **Correct answer: A**

Rationale
- **A — Correct.** The **most stable** client — post-op, eating, ambulating — is the safest to discharge or transfer to open a bed for incoming casualties.
- **B — Incorrect.** A client on **vasopressors** is unstable and cannot be moved out.
- **C — Incorrect.** New chest pain with pending enzymes is potentially unstable (possible MI) and must stay.
- **D — Incorrect.** High fever with rising WBCs suggests active infection/possible sepsis — not appropriate to discharge.

📝 **NCLEX Tip.** "Who can you discharge/transfer?" flips normal priority: you pick the **most stable** client, not the sickest. Read whether the stem wants *who to see first* vs *who can leave.*

💡 **Clinical Pearl / Memory Trick.** In disaster/staffing questions, **the boring, stable client is the right answer** — they can safely go. Keep the unstable ones close.

Question 45 — Safe medication administration

Category: Safety / Pharmacology · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Easy

Scenario. A nurse is preparing to administer a medication and notices the prescription is difficult to read and the dose seems unusually high.

What should the nurse do?

- **A.** Administer the dose as written to avoid delaying treatment.
- **B.** Give a smaller dose that seems safer based on judgment.
- **C.** Hold the medication and clarify the order with the prescriber.
- **D.** Ask another nurse to interpret the handwriting and proceed.

🎨 *Suggested illustration:* a nurse pausing with a med cup in hand, phone to ear calling the prescriber, an unclear order highlighted with a question mark.

✅ **Correct answer: C**

Rationale
- **A — Incorrect.** Administering an unclear or possibly excessive dose risks serious harm — never give what you can't verify.
- **B — Incorrect.** Improvising a dose is outside the nurse's scope and unsafe.
- **C — Correct.** When an order is **illegible, incomplete, or appears incorrect**, the nurse **holds the medication and clarifies directly with the prescriber** before giving anything.
- **D — Incorrect.** A second nurse's guess doesn't make an unverified order safe; the prescriber must clarify.

📝 **NCLEX Tip.** The nurse is the **last safety check.** Any unclear, incomplete, or unsafe-looking order is **clarified before administration** — never assumed.

💡 **Clinical Pearl / Memory Trick. "If it isn't right, don't write (or give).**" Remember the **rights** of medication administration — right client, drug, dose, route, time, documentation, reason, response.

Question 46 — Anaphylaxis

Category: Clinical Judgment / Emergency · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. Minutes after receiving an IV antibiotic, a client develops facial swelling, hives, wheezing, and a drop in blood pressure.

What is the nurse's priority action?

- **A.** Document the reaction and continue the infusion at a slower rate.
- **B.** Stop the antibiotic, maintain the airway, and prepare to administer epinephrine.
- **C.** Administer an oral antihistamine and reassess in 20 minutes.
- **D.** Apply a cool compress to the swollen areas.

🎨 *Suggested illustration:* an IV clamped shut, a client with facial/lip swelling and hives, an epinephrine auto-injector highlighted and ready.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Continuing the trigger (even slower) worsens a life-threatening reaction.
- **B — Correct.** This is **anaphylaxis.** The nurse **stops the causative drug, protects the airway, and gives epinephrine** — the first-line, life-saving medication — followed by oxygen, IV fluids, antihistamines, and corticosteroids.
- **C — Incorrect.** Oral antihistamines are too slow and not first-line for anaphylaxis with airway/BP compromise.
- **D — Incorrect.** A compress does nothing for airway swelling or shock.

📝 **NCLEX Tip.** Anaphylaxis first-line = **epinephrine**, every time. Antihistamines and steroids are **adjuncts**, not the priority.

💡 **Clinical Pearl / Memory Trick.** "**Epi first, and don't rehearse.**" Stop the trigger → **epinephrine** → airway/oxygen → fluids for the pressure.

Question 47 — Acute chest pain

Category: Clinical Judgment / Cardiac · **Client Need:** Physiological Adaptation · **Difficulty:** Moderate

Scenario. A client on a med-surg unit suddenly reports crushing substernal chest pressure radiating to the left arm, with nausea and diaphoresis.

Which is the nurse's first action?

- **A.** Call the client's family to notify them of the change.
- **B.** Stay with the client, call for help/activate the response, and prepare to give oxygen and obtain an ECG.
- **C.** Have the client walk to the nurses' station for evaluation.
- **D.** Give a full glass of water and wait to see if the pain resolves.

🎨 *Suggested illustration:* a client clutching the chest with pain radiating down the left arm, a nurse pressing the call button while reaching for an oxygen line and ECG leads.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** Notifying family is not the immediate priority during a possible heart attack.
- **B — Correct.** Suspected **acute coronary syndrome** demands the nurse **stay with the client, summon help/activate the rapid response or protocol, apply oxygen if indicated, obtain a 12-lead ECG, and anticipate aspirin, nitroglycerin, and continuous monitoring.**
- **C — Incorrect. Never ambulate** a client with possible cardiac chest pain — exertion increases oxygen demand and risk.
- **D — Incorrect.** Water and watchful waiting delay life-saving care.

📝 **NCLEX Tip.** Cardiac chest pain = **rest + oxygen + ECG + meds**, and keep the client **still.** Any answer that makes the client exert is wrong.

💡 **Clinical Pearl / Memory Trick.** Remember the classic bundle **"MONA"** — **M**orphine, **O**xygen, **N**itroglycerin, **A**spirin — while an ECG is obtained (aspirin is often given first/early). Don't walk the chest-pain client.

Question 48 — Recognizing early deterioration

Category: Clinical Judgment · **Client Need:** Reduction of Risk Potential · **Difficulty:** Hard

Scenario. A postoperative client's vital signs over three hours show a gradually rising heart rate (88 → 118/min), a rising respiratory rate (16 → 26/min), a falling blood pressure (122/78 → 100/60), and new restlessness.

How should the nurse interpret these trends?

- **A.** Expected postoperative recovery — continue routine monitoring.
- **B.** Early signs of clinical deterioration that warrant escalation.
- **C.** Signs of adequate pain control taking effect.
- **D.** Normal variation that requires no documentation.

🎨 *Suggested illustration:* a small trend chart with HR and RR lines climbing while BP drifts down, and a "call for help" flag rising at the end.

✅ **Correct answer: B**

Rationale
- **A — Incorrect.** A steadily rising HR/RR with falling BP is **not** normal recovery — it points to a developing problem (bleeding, sepsis, shock).
- **B — Correct.** Reading the **trend**, not single numbers, reveals **early deterioration.** The nurse escalates — reassesses, notifies the provider or activates the **rapid response team** — before the client crashes.
- **C — Incorrect.** Effective pain control would not cause rising respiratory rate, tachycardia, and hypotension together.
- **D — Incorrect.** These changes are significant and must be documented and acted upon.

📝 **NCLEX Tip.** Next Gen NCLEX loves **trends over time.** Up-HR + up-RR + down-BP + new restlessness = **compensating shock** — recognize and **escalate early.**

💡 **Clinical Pearl / Memory Trick. "The story is in the trend."** Restlessness and a climbing heart/resp rate are often the body's **first quiet alarm** before pressure ever drops far.

Question 49 — Oxygen and fire safety

Category: Safety · **Client Need:** Safe and Effective Care Environment · **Difficulty:** Easy

Scenario. A nurse provides safety teaching to a client using home oxygen.

Which client statement indicates correct understanding?

- **A.** "I can smoke as long as I lower the oxygen flow first."
- **B.** "I'll keep the oxygen at least several feet away from stoves, candles, and open flames."
- **C.** "I'll use petroleum jelly on my lips to keep them from drying out."
- **D.** "Wool blankets are best for reducing static around the oxygen."

🎨 *Suggested illustration:* a home oxygen concentrator with a "no smoking / no open flame" sign, positioned well away from a kitchen stove.

✅ **Correct answer: B**

Rationale
- **A — Incorrect. No smoking near oxygen — ever.** Oxygen supports combustion and can cause a flash fire regardless of flow rate.
- **B — Correct.** Oxygen must be kept **away from flames, heat, and sparks** (stoves, candles, heaters). "No smoking" signs and safe distances are standard.
- **C — Incorrect. Petroleum-based products are flammable** near oxygen — use water-based lip moisturizer instead.
- **D — Incorrect. Wool and synthetics build static/sparks**; cotton is preferred around oxygen.

📝 **NCLEX Tip.** Oxygen safety = **no flames, no petroleum products, no static-prone fabrics.** Any answer that allows smoking near oxygen is wrong.

💡 **Clinical Pearl / Memory Trick.** "**Oxygen doesn't burn — it makes everything else burn faster.**" Keep flames, grease (petroleum), and sparks far away.

Question 50 — Clinical judgment capstone (bow-tie style)

Category: Clinical Judgment · **Client Need:** Physiological Adaptation · **Difficulty:** Hard

Scenario. A client 1 day after abdominal surgery reports increasing thirst and dizziness. Assessment: heart rate 124/min, blood pressure 92/58 mm Hg, urine output 15 mL over 2 hours, and a saturated abdominal dressing with bright red drainage.

The nurse determines the client is most likely experiencing *(1)* and should first *(2)*.

(1) Condition options: fluid volume overload · **hypovolemic shock** · allergic reaction · normal postoperative fluid shift

(2) First action options: apply firm pressure to the site and notify the surgeon while increasing IV fluids · slow the IV fluids to prevent overload · administer the next dose of pain medication · reposition and recheck in one hour

🎨 *Suggested illustration:* a bow-tie layout — center "problem," left "supporting data" (low BP, high HR, low urine, bleeding dressing), right "actions" (pressure, notify, fluids).

✅ **Correct answers: (1) hypovolemic shock · (2) apply firm pressure to the site and notify the surgeon while increasing IV fluids**

Rationale

- **Condition — hypovolemic shock (correct).** Tachycardia, hypotension, very low urine output, thirst, dizziness, and **active bright-red bleeding** together point to blood/volume loss — **hypovolemic (hemorrhagic) shock.**
- *Fluid overload* would show high BP, crackles, and edema — not bleeding and low output from hypoperfusion.
- *Allergic reaction* would show hives, swelling, and wheezing.
- *Normal fluid shift* does not include a saturated bloody dressing with unstable vitals.
- **First action — pressure + notify + fluids (correct). Control the bleeding** (direct pressure), **escalate** to the surgeon, and **restore volume** with IV fluids.
- *Slowing fluids* would worsen shock.
- *Pain medication* can further lower blood pressure and ignores the hemorrhage.
- *Repositioning and waiting* dangerously delays care.

📝 **NCLEX Tip.** Bow-tie items test the full clinical-judgment loop: **recognize cues → analyze → prioritize the problem → act → (later) evaluate.** Let the data in the stem point you to the condition, then choose the action that treats *that* condition.

💡 **Clinical Pearl / Memory Trick.** Shock is about **perfusion**: **fast pulse, low pressure, low urine, cool and thirsty.** For **hypovolemic** shock: **stop the loss and replace the volume —** in that order.

You finished all 50. 🎉

That's the whole workbook — every Next Gen item type, with the reasoning laid bare. If a rationale surprised you, that's the win: you just closed a gap the test would have found.

Keep going in the app: tag the ones you missed as *Learning*, and Esi will bring them back at exactly the right time.

— *The Must Love Scrubs team*

M❤️‍🩹S